PATIENT HANDOUT

Pelvic Floor Exercises and Bladder Training for Women

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Who this handout is for

These exercises and habits can help women who have:

- **leaking** when they cough, sneeze, laugh, lift or exercise;
- **urgency**: a sudden, strong need to pee that is hard to hold;
- needing to pee **very often**, day or night;
- weak pelvic floor muscles **after childbirth** or **after menopause**.

Check with your doctor first if:

- you have **pelvic pain**, pain with sex, or trouble inserting a tampon. Squeezing exercises can make these worse. Ask about our guide *Releasing Pelvic Pain* instead.
- you have trouble **emptying your bladder**.

Do not do these exercises while you have a **catheter** in.

Your pelvic floor

The pelvic floor is a sling of muscles at the bottom of your pelvis. It runs from your tailbone at the back to your pubic bone at the front.

These muscles:

- squeeze to **hold in urine, stool and gas**;
- relax to let you **pee and have a bowel movement**;
- **hold up** your bladder, uterus and bowel.

Like any muscle, they get stronger with exercise, at any age.

Step 1: Find the right muscles

This is the most important step. Many women squeeze the wrong muscles at first.

Sit or lie down. Relax your belly, buttocks and thighs. Then try one of these:

1. **Hold in gas.** Pretend you are trying to stop yourself from passing gas. Squeeze and lift the muscles around your anus (bum).
2. **Stop the flow.** Pretend you are trying to stop peeing midstream.
3. **Pick up a marble.** Imagine you are sitting on a marble and gently lifting it up inside you with your vaginal muscles.

You should feel a squeeze around the vagina and anus, and a **lift** upward and inward.

How to check:

- Put one clean finger just inside your vagina and squeeze. You should feel a gentle tightening and lift around your finger.
- Or look with a hand mirror. The area between the vagina and anus should draw **in and up**, not push out.

Don't:

- hold your breath. Breathe normally the whole time;
- squeeze your buttocks or thighs;
- pull your belly in hard;
- push down, as if having a bowel movement.

It is fine to try stopping your urine **once**, to learn the feeling. **Do not** make it a habit. Stopping your flow often can stop your bladder from emptying properly.

Step 2: Do the exercises

There are two kinds of squeeze. Do both, **3 times a day**, for the first 12 weeks. After that, once a day keeps your muscles strong.

Long squeezes

These build strength and help you hold on.

1. Squeeze and lift.
2. **Hold** while you count (the table below shows how long).
3. **Let go completely** and rest for the same count.
4. Repeat **10 times**.

Quick squeezes

These help you stop leaks from a cough, sneeze or sudden urge.

1. Squeeze and lift **quickly**.
2. Let go right away.
3. Rest for 2 seconds.
4. Repeat **10 times**.

Tips

- If you can’t hold for the full count yet, hold as long as you can. You will get better.
- Resting fully between squeezes is as important as squeezing.
- If your muscles get tired and the squeeze fades, stop and try again later.
- Link the exercises to things you do every day: after brushing your teeth, during the TV news, at red lights.
- Don’t do the exercises while you are peeing.

Your program

Start easy. Build up slowly.

Weeks	Position	Long squeezes	Quick squeezes
1 and 2	Lying down	Hold 3 seconds, rest 3 seconds. 10 times.	10 times
3 and 4	Lying or sitting	Hold 5 seconds, rest 5 seconds. 10 times.	10 times
5 to 8	Sitting	Hold 8 seconds, rest 8 seconds. 10 times.	10 times
9 to 12	Standing	Hold 10 seconds, rest 10 seconds. 10 times.	10 times
After 12 weeks	Any position	Hold 10 seconds. 10 times, **once a day, for life**.	10 times

Do all of this 3 times a day (morning, afternoon and evening), until week 12.

Step 3: Squeeze before you sneeze

Once you can find your muscles, use them in daily life. Squeeze and hold your pelvic floor **just before** and **during**:

- a cough or sneeze;
- a laugh;
- lifting something;
- standing up from a chair;
- bending over;
- getting out of the car.

With practice, this becomes automatic. Many people find this simple trick stops many leaks on its own.

How long until it works?

- Most people notice a difference in **6 to 12 weeks**.
- The full benefit can take **3 to 6 months**.
- If you stop, the muscles weaken again. Keep doing them once a day, for life.

Pregnancy, childbirth and menopause

During pregnancy

Pelvic floor exercises are safe during pregnancy and help prevent leaking later on. Start any time.

After childbirth

You can usually start gentle squeezes a few days after delivery, after a vaginal birth or a caesarean, once it is comfortable. Start at week 1 of the program. If you had a large tear or still leak at your six-week check, ask about seeing a pelvic floor physiotherapist.

After menopause

Lower estrogen levels after menopause can make the tissues around the bladder and vagina thinner and drier. This can make leaking, urgency and

bladder infections more likely. A small amount of **vaginal estrogen** (a cream, tablet or ring) can help. Ask your doctor if it is right for you.

A feeling of pressure or a bulge

Some women feel heaviness, dragging or a **bulge** in the vagina, often worse at the end of the day. This can be a **prolapse**, where the bladder, uterus or bowel sags down. Pelvic floor exercises can help with mild prolapse.

Tell your doctor if you notice a bulge. A pelvic floor physiotherapist can help, and a small support device called a **pessary** is another option.

How to sit on the toilet

- **Sit all the way down.** Don't hover over the seat. Hovering tightens the pelvic floor and stops the bladder from emptying fully.
- Keep your feet flat on the floor, or on a small stool.
- Lean forward a little, with your elbows on your knees.
- Relax and let the urine flow. Don't push.

Bladder retraining

If you have **urgency** or need to go **very often**, your bladder may have learned to send "I need to go" signals too early. A healthy bladder can usually wait **3 to 4 hours** between trips during the day. Bladder retraining teaches your bladder to hold more and calm down. It works best together with your pelvic floor exercises.

1. **Keep a bladder diary for 3 days.** Use the diary at the end of this handout. Write down each time you pee and each time you drink. This shows how long you usually wait between trips.

2. **Pick your starting time.** Take the time you usually wait, and add 15 minutes. For example, if you usually go every hour, start with every 1 hour and 15 minutes. If you are not sure, bring your diary to us and we will help you choose.
3. **Go by the clock, not the urge.** During the day, try to pee only at your planned times. If it is time and you don't feel the need, go anyway.
4. **When the urge comes early, use the urge control steps below.** Most urges fade within a minute or two.
5. **Add 15 minutes each week** once you can manage your current time on most days. Keep going until you can wait 3 to 4 hours.

An example

Week	Time between trips
1	1 hour 15 minutes
2	1 hour 30 minutes
3	1 hour 45 minutes
4	2 hours
Keep going	Add 15 minutes a week until you reach 3 to 4 hours

- Follow the schedule only when you are **awake**. At night, go when you wake up and need to.
- Bad days are normal. If you have a hard week, go back to your last comfortable time and build up again.
- Retraining usually takes **6 to 12 weeks**.

Urge control: what to do when the urge hits

When you feel a strong urge, **don't rush** to the bathroom. Rushing makes the bladder squeeze harder and makes leaks more likely. Instead:

1. **Stop.** Stand still, or sit down if you can.
2. **Squeeze.** Do 5 strong, quick pelvic floor squeezes. This sends a "wait" message to your bladder.
3. **Breathe.** Take slow, relaxed breaths. Let your shoulders drop.
4. **Think of something else.** Count backward from 100, or name the people in your family.
5. **Wait for the urge to fade.** Then walk calmly to the bathroom, or wait until your planned time.

Some people also find it helps to curl their toes, or to sit on a firm chair.

Habits that help your bladder

Drink plenty of water

- Most adults need about **6 to 8 cups (1.5 to 2 litres)** of fluid a day, and more in hot weather or when active. Water is best.
- Aim for **pale yellow** urine. Dark urine means you need to drink more.
- **Don't cut back on water to avoid leaks.** Strong, concentrated urine irritates the bladder and makes urgency worse. Too little water also causes constipation.
- **Spread your drinks** through the day. Small amounts often are better than a lot at once.
- Drink most of your fluids **earlier in the day**, and slow down in the **2 to 3 hours before bed**.
- If your diary shows you drink **much more** than this (more than about 10 cups or 2.5 litres a day), cutting back to 6 to 8 cups can help you go less

often.

If you have heart, kidney or liver disease, have been told your blood sodium (salt) is low, or your doctor has told you to limit how much you drink, ask your doctor before drinking more. Follow their advice.

Cut back on drinks that bother the bladder

These can make urgency and frequency worse:

- coffee and tea (including decaf for some people);
- cola and energy drinks;
- alcohol;
- fizzy drinks;
- diet drinks and artificial sweeteners;
- citrus juices.

Try cutting back for 2 to 3 weeks and see if your bladder settles. If you drink a lot of coffee, cut down slowly to avoid headaches.

Keep your bowels soft and regular

Constipation presses on your bladder, and straining weakens your pelvic floor.

- Eat more **fibre**: vegetables, fruit, whole grains, beans, lentils, prunes and ground flaxseed. Add fibre slowly to avoid gas.
- Drink plenty of water. Fibre works best with water.
- Stay active. A daily walk helps keep your bowels moving.
- Go when you feel the urge. Don’t put it off.
- Put your **feet on a small stool** so your knees are higher than your hips. Lean forward and relax your belly.
- **Don’t strain** or hold your breath. Breathe out slowly as you go.
- If you are still constipated, ask your doctor or pharmacist about a gentle daily laxative such as **polyethylene glycol** (for example RestoraLAX or Lax-A-Day), or a fibre supplement such as psyllium. Always take fibre supplements with a full glass of water.

Other things that help

- **Keep a healthy weight.** Extra weight presses on the bladder. Losing even a small amount can reduce leaking.
- **Stop smoking.** Smoking causes coughing, which strains the pelvic floor, and it irritates the bladder.
- **Treat a long-lasting cough** with your doctor's help.
- **Empty your bladder fully.** When you finish peeing, wait 20 to 30 seconds, lean forward, and try again. This is called "double voiding."
- **Avoid extra "just in case" trips** between your planned times, such as every time you pass a bathroom. They teach your bladder to hold less.
- **Review your medications** with your doctor or pharmacist. Some, such as "water pills" (diuretics), make you pee more. Taking them in the morning may help.

Getting up at night

- Slow down your drinks in the 2 to 3 hours before bed. Avoid alcohol and caffeine in the evening.
- Pee just before you go to bed.
- If your ankles swell during the day, put your **feet up** for an hour in the late afternoon. This helps your body clear extra fluid before bedtime instead of overnight.
- Loud snoring or stopping breathing at night (sleep apnea) can cause night-time peeing. Tell your doctor if you snore heavily.
- Keep a clear, lit path to the bathroom to prevent falls.

When to get help

Call your doctor if you have:

- burning when you pee, fever, or cloudy, bad-smelling urine;
- **blood** in your urine;
- trouble passing urine, or a feeling that your bladder won't empty;
- pain during the exercises;
- leaking that is getting worse;
- no improvement after **3 months** of regular exercises;
- blood from your bottom, black stools, or a change in your bowel habits that lasts more than a few weeks.

If you **cannot pee at all** and your bladder feels full and painful, go to the **emergency department**.

Also go to the **emergency department** if you suddenly start leaking urine or stool together with **back pain**, **numbness** between your legs, or **weakness** in your legs.

A pelvic floor physiotherapist can help

Pelvic floor physiotherapists are specially trained to check these muscles and coach you through the exercises. People who see one often improve faster. Consider seeing one if:

- you are not sure you are squeezing the right muscles;
- you are not getting better after 3 months;
- you have had prostate surgery, a difficult childbirth, or a prolapse;
- you leak stool or gas.

You don't need a doctor's referral. Private clinics are usually not covered by OHIP, but workplace and retiree health plans often pay part. If you are **65 or older**, ask about OHIP-funded physiotherapy at community clinics. Our office can suggest clinics.

My exercise log

Put a check mark each time you do your exercises.

Week	Mon	Tue	Wed	Thu	Fri	Sat	Sun
1							
2							
3							
4							
5							
6							
7							
8							
9							
10							
11							
12							

Weeks 1 to 12: aim for 3 check marks a day (morning, afternoon, evening). After week 12: once a day.

For education only. This handout does not replace advice from your own care team. October 2026. Find this handout online at nicholaspowerurology.com/education/kegels

My bladder diary

Fill in one page for each day, for 3 days. Bring it to your next appointment.

Day: _____ Date: _____ Time I got up: _____ Time I went to bed: _____

Time	What I drank, and how much	Peed? (✓)	Urge? (0–3)	Leak? (small / large)	What I was doing

Urge: 0 = none, 1 = mild, 2 = strong, 3 = had to rush. **Pads used today:** _____

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